

Once-daily Oral Zasocitinib Demonstrates Rapid and Reproducible Skin Clearance with a Consistent Safety Profile in Moderate-to-Severe Plaque Psoriasis: Results from Two Randomized Phase 3 Trials (LATITUDE-PsO-3001 and 3002)

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Disclosures

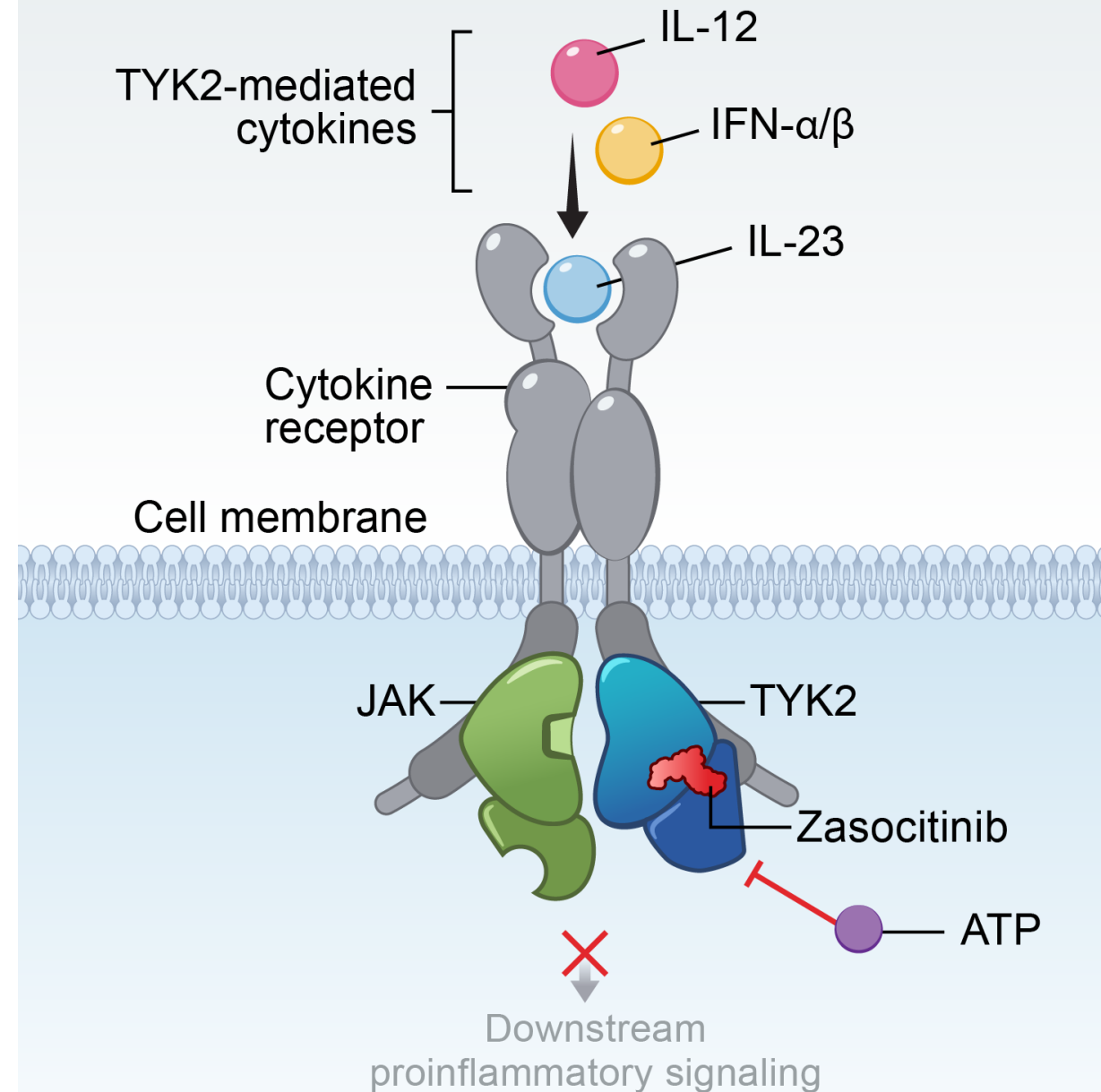
Dr Melinda Gooderham has been an investigator, speaker and/or advisor for: AbbVie, Acelyrin, Akros, Alumis, Amgen, AnaptysBio, Apogee, Arcutis, Aristea, Bausch Health, Bristol Myers Squibb, Boehringer Ingelheim, Dermavant, Dermira, Eli Lilly, Galderma, GSK, Incyte, Inmagene, JAMP Pharma, Janssen, L’Oreal, LEO Pharma, MedImmune, Meiji, Moonlake, Nektar, Nimbus, Novartis, Organon, Oruka, Pfizer, Q32 Bio, Regeneron, Sanofi Genzyme, Sun Pharma, Takeda, Tarsus, UCB, Union, Ventyx and Vyne

Zasocitinib is an investigational, oral, allosteric, highly selective and potent TYK2 inhibitor^{1,2}

- More than 1 millionfold greater binding selectivity for TYK2 versus JAK1, JAK2 and JAK3^{1,2}
- Maintains 24-hour inhibition of IL-23 plus other core disease-driving immune pathways^{2,3}
- Well tolerated and efficacious in a phase 2b trial in patients with moderate-to-severe plaque psoriasis⁴

Objective:

To evaluate the efficacy and safety of zasocitinib in adults with moderate-to-severe plaque psoriasis in two pivotal phase 3 studies: LATITUDE-PsO-3001 (NCT06088043) and LATITUDE-PsO-3002 (NCT06108544)



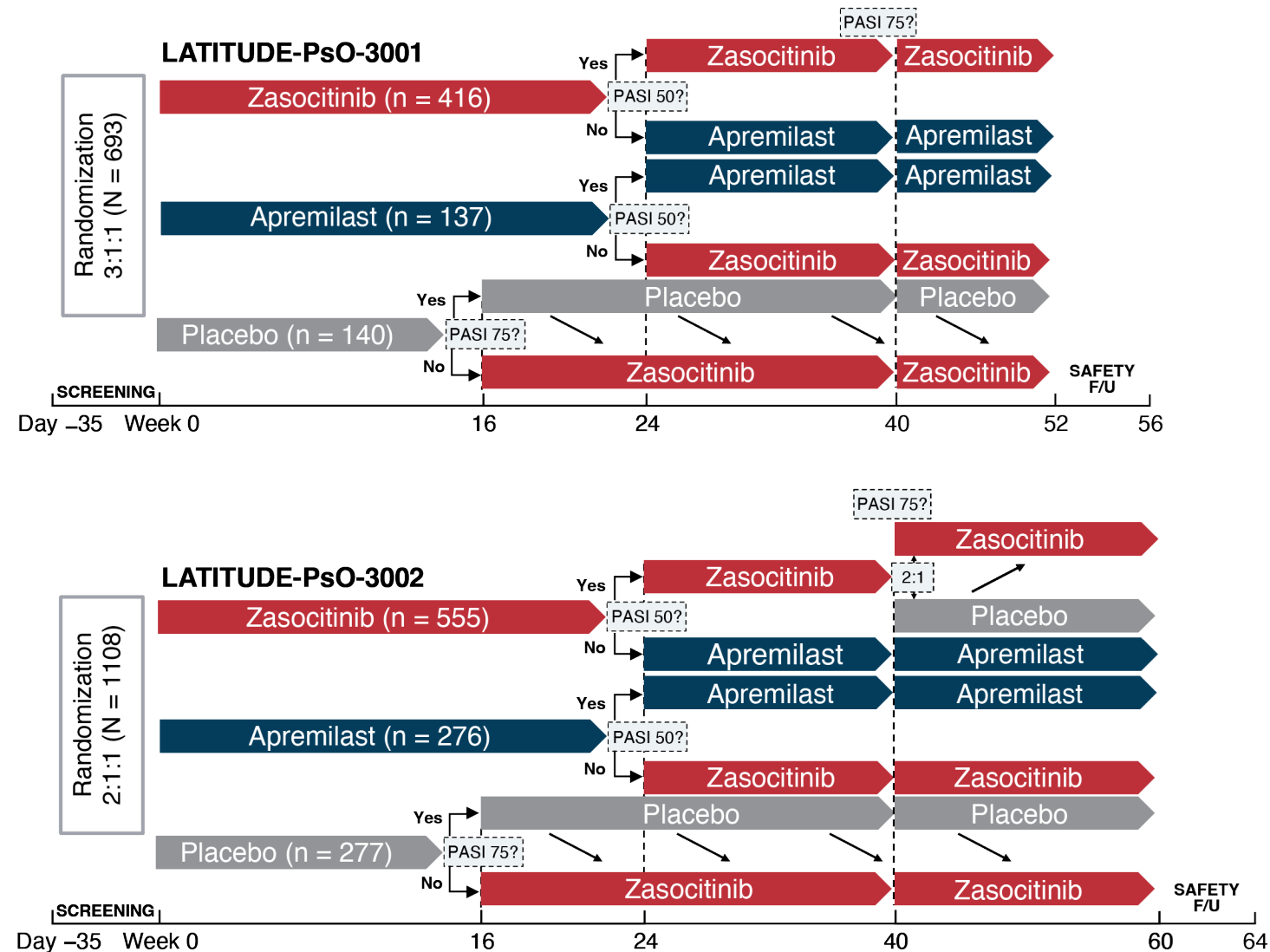
ATP, adenosine triphosphate; IFN, interferon; IL, interleukin; JAK, Janus kinase; TYK, tyrosine kinase.

1. Leit S *et al. J Med Chem* 2023;66:10473–96; 2. Mehrotra S *et al. J Invest Dermatol* 2025;146:214–22;

3. Rusiñol L and Puig L. *Int J Mol Sci* 2023;24:3391. 4. Armstrong A *et al. JAMA Dermatol* 2024;160:1066–74.

Presented at the American Academy of Dermatology (AAD) 2026 Annual Meeting; March 27–31, 2026; Denver, Colorado, USA. Copyright © 2026 Takeda Pharmaceutical Company Limited. All rights reserved. TAKEDA and the TAKEDA logo are registered trademarks of Takeda Pharmaceutical Company Limited. For personal use only.

LATITUDE-PsO-3001 and 3002 were randomized, multicenter, double-blind, placebo- and apremilast-controlled phase 3 trials



Eligibility

- Adults (≥ 18 years)
- Plaque psoriasis diagnosis for ≥ 6 months prior to screening
- PASI ≥ 12 , sPGA ≥ 3 , $\geq 10\%$ BSA
- Candidate for phototherapy or systemic therapy

Endpoints

Co-primary endpoints at Week 16 (versus placebo):

- **sPGA 0/1^a**
- **PASI 75**

Key secondary endpoints (versus placebo or apremilast) included:

- PASI 75 at Week 4
- sPGA 0, PASI 75/90/100 or DLQI 0/1^b at Week 16 or 24
- sPGA 0/1 or PASI 75 maintenance at Week 60

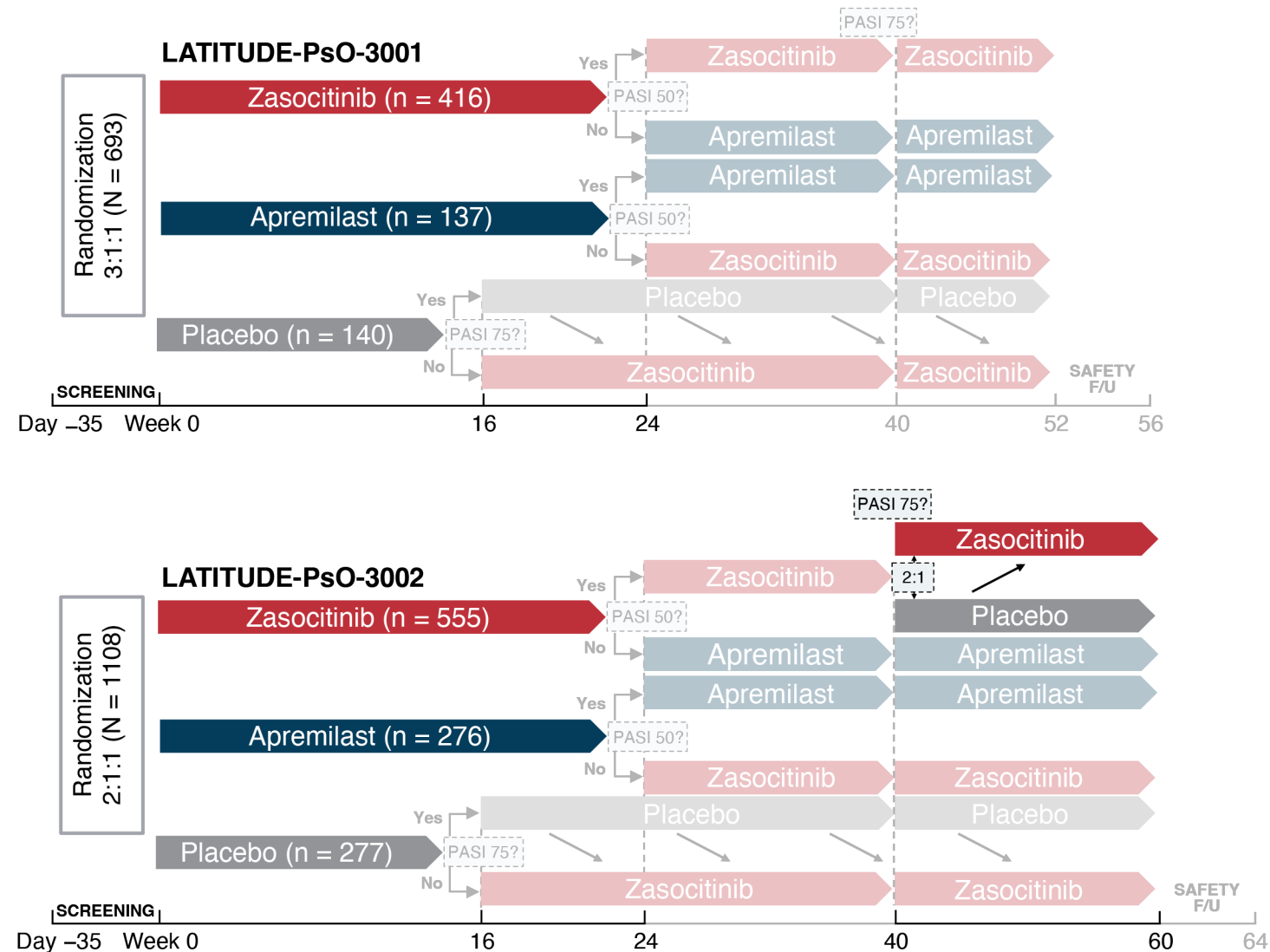
Safety endpoints included:

- TEAEs; laboratory parameters

^aWith a ≥ 2 -point decrease from baseline. ^bWith a baseline DLQI score ≥ 2 .

BSA, body surface area; DLQI, Dermatology Life Quality Index; F/U, follow-up; PASI, Psoriasis Area and Severity Index; sPGA, static Physician's Global Assessment; TEAE, treatment-emergent adverse events.

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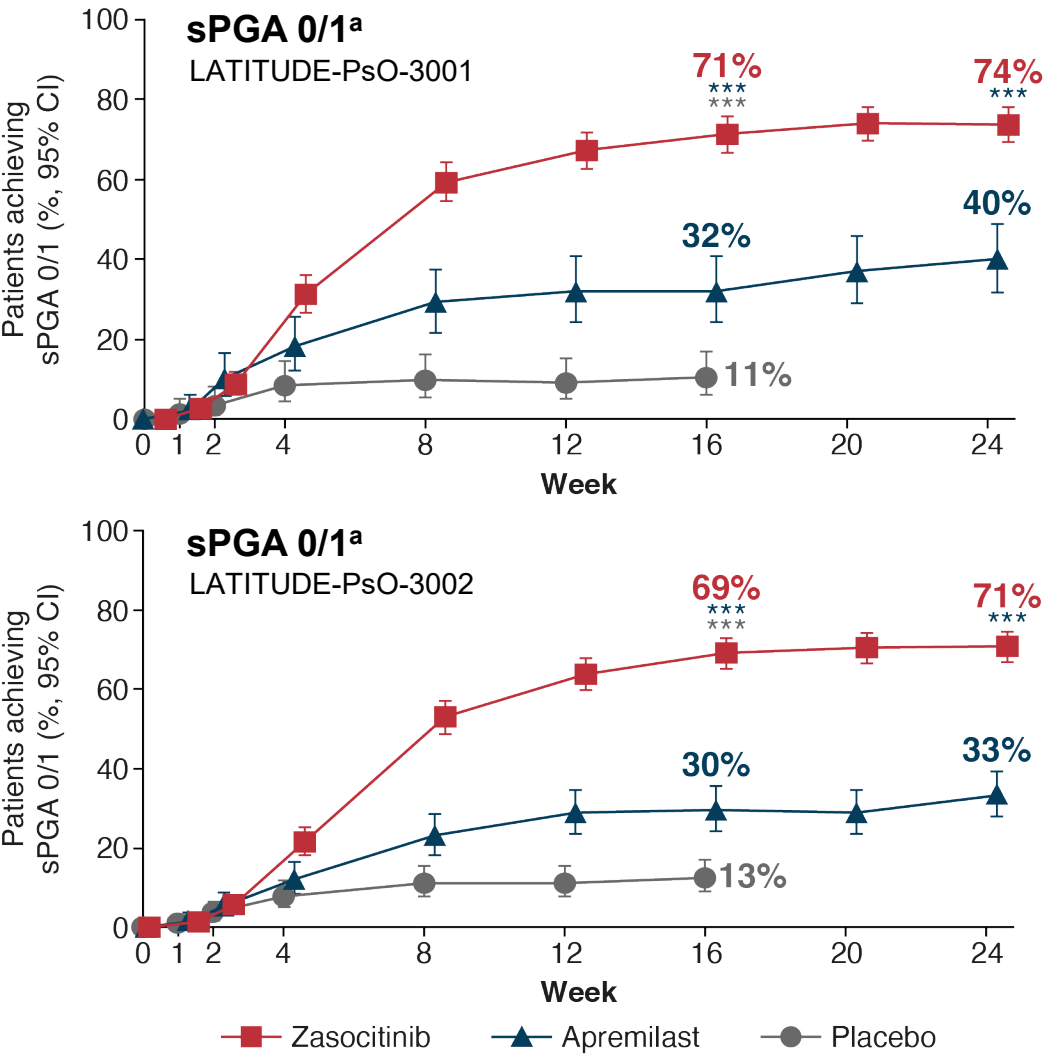
BSA, body surface area; DLQI, Dermatology Life Quality Index; F/U, follow-up; PASI, Psoriasis Area and Severity Index; sPGA, static Physician's Global Assessment; TEAE, treatment-emergent adverse events.

Baseline demographics and characteristics were generally similar across treatment arms in each study

	LATITUDE-PsO-3001			LATITUDE-PsO-3002		
	Zasocitinib (n = 416)	Apremilast (n = 137)	Placebo (n = 140)	Zasocitinib (n = 555)	Apremilast (n = 276)	Placebo (n = 277)
Age, years	43.8 (13.26)	46.0 (14.10)	45.3 (13.54)	45.8 (13.33)	46.1 (13.37)	46.5 (13.19)
Sex, male, n (%)	295 (70.9)	95 (69.3)	93 (66.4)	367 (66.1)	187 (67.8)	188 (67.9)
Race, White, n (%)	255 (61.3)	88 (64.2)	87 (62.1)	472 (85.0)	233 (84.4)	240 (86.6)
BMI, kg/m ²	29.7 (6.8)	28.5 (6.3)	28.2 (6.5)	30.2 (6.8)	30.1 (6.5)	30.4 (7.2)
Psoriasis duration, median (range) years ^a	13.6 (0.6–62.4)	14.0 (0.6–71.3)	12.5 (0.6–59.3)	15.1 (0.5–69.2)	16.5 (0.6–60.5)	15.1 (0.6–65.9)
PASI score	19.7 (7.5)	20.5 (9.0)	20.3 (7.4)	21.3 (9.3)	21.4 (8.6)	21.1 (8.5)
sPGA score						
3 (moderate), n (%)	329 (79.1)	116 (84.7)	112 (80.0)	473 (85.2)	237 (85.9)	228 (82.3)
4 (severe), n (%)	85 (20.4)	21 (15.3)	28 (20.0)	80 (14.4)	39 (14.1)	48 (17.3)
BSA, %	24.0 (14.0)	25.8 (15.9)	24.2 (14.4)	27.9 (17.8)	27.8 (16.5)	27.0 (16.3)
DLQI score ^b	12.7 (7.2)	11.1 (6.5)	12.2 (7.3)	11.6 (7.2)	11.5 (6.7)	12.2 (7.4)
Bio-experienced, n (%)	141 (33.9)	40 (29.2)	45 (32.1)	155 (27.9)	80 (29.0)	83 (30.0)

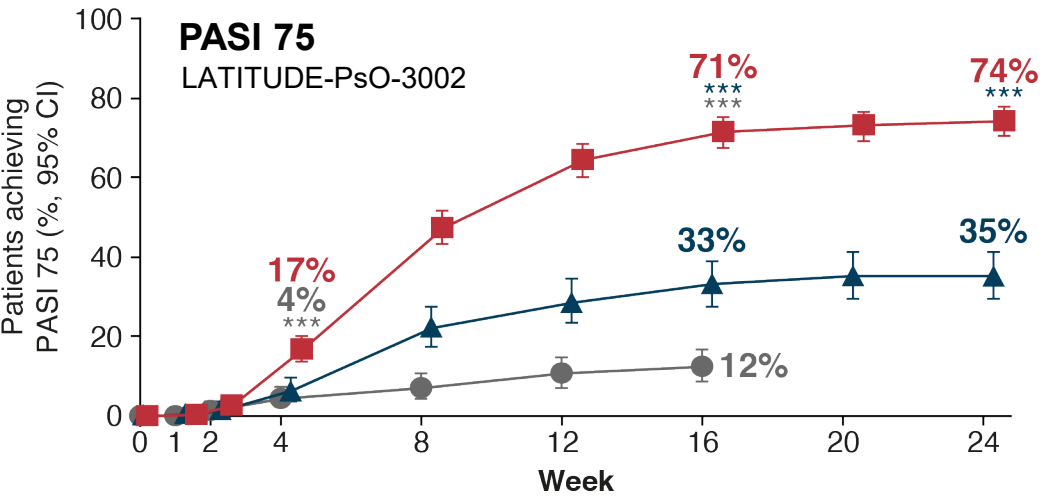
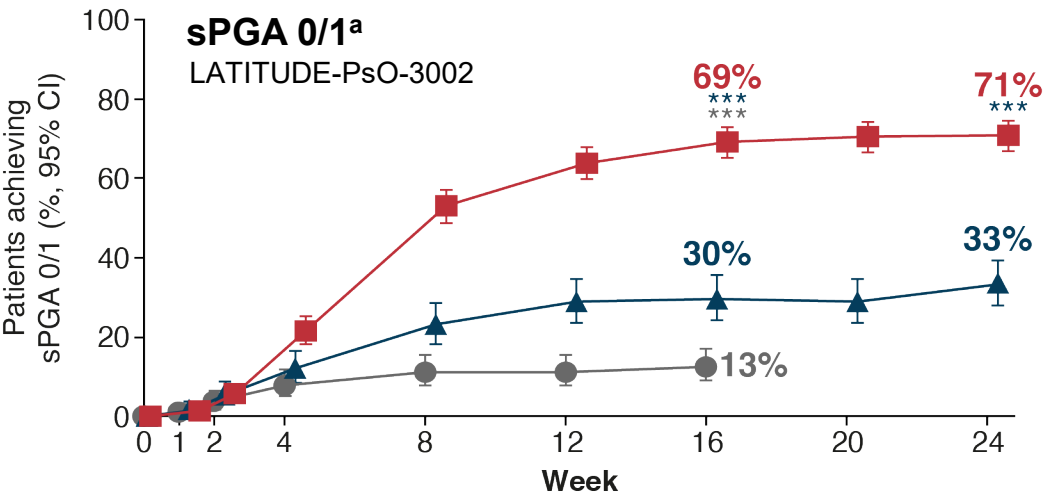
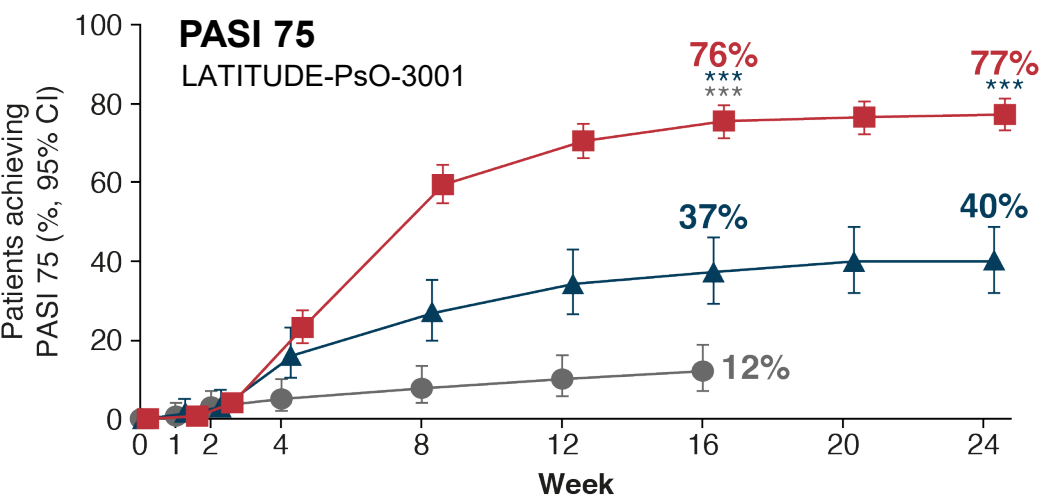
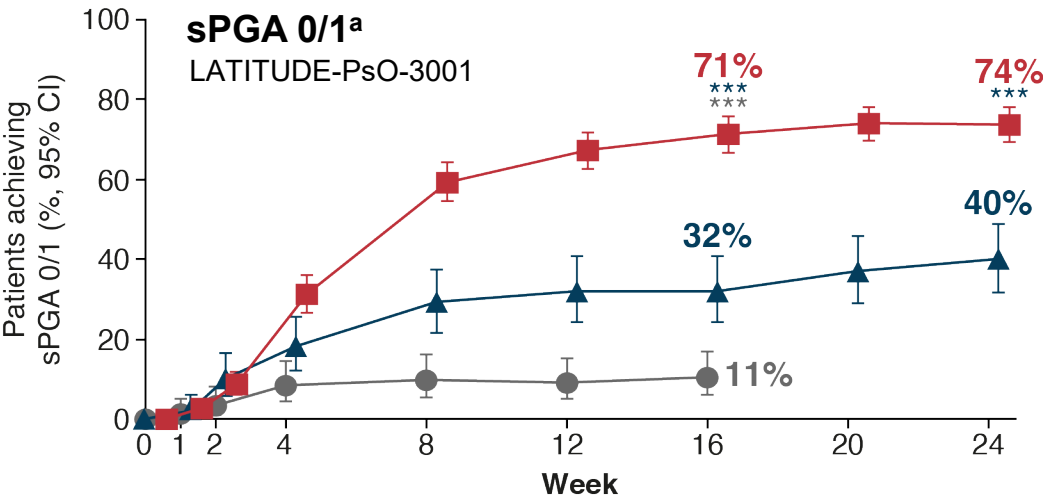
Data are mean (SD) unless otherwise stated. ^aData missing for one patient receiving zasocitinib (LATITUDE-PsO-3002). ^bData missing for three patients receiving zasocitinib and one receiving apremilast (LATITUDE-PsO-3001), and for three patients receiving zasocitinib, one patient receiving apremilast, and four patients receiving placebo (LATITUDE-PsO-3002). BMI, body mass index; BSA, body surface area; DLQI, Dermatology Life Quality Index; PASI, Psoriasis Area and Severity Index; SD, standard deviation; sPGA, static Physician's Global Assessment.

Zasocitinib met the co-primary endpoints in both studies (sPGA 0/1 and PASI 75 versus placebo at Week 16)



^aWith a ≥ 2-point decrease from baseline. Number of patients based on the full analysis set with non-responder imputation. LATITUDE-PsO-3001: zasocitinib (n = 416), apremilast (n = 137), placebo (n = 140). LATITUDE-PsO-3002: zasocitinib (n = 555), apremilast (n = 276), placebo (n = 277). *P* values for comparison versus apremilast (in blue) and versus placebo (in gray) based on a stratified Cochran–Mantel–Haenszel test: ****p* < 0.001. CI, confidence interval; PASI, Psoriasis Area and Severity Index; sPGA, static Physician’s Global Assessment.

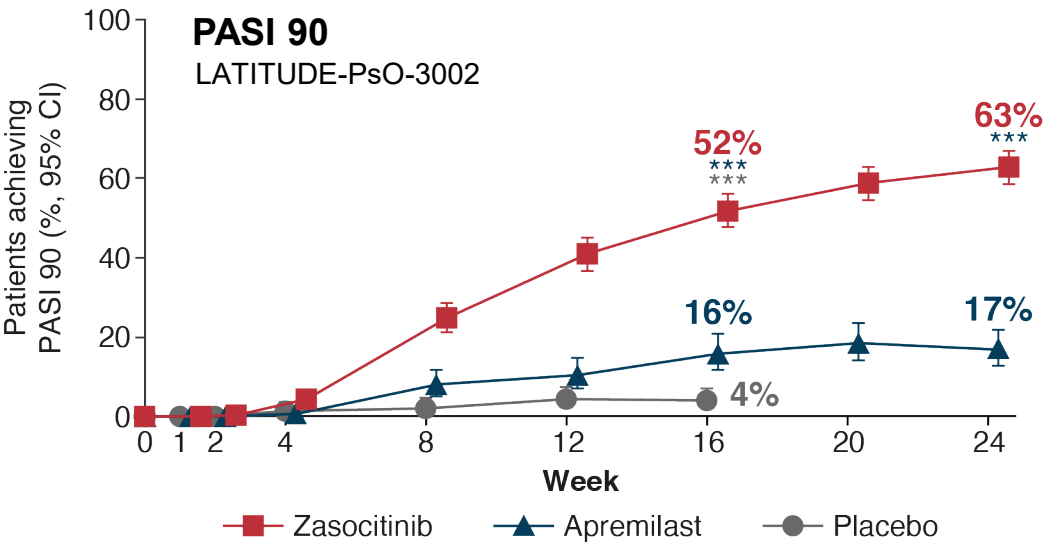
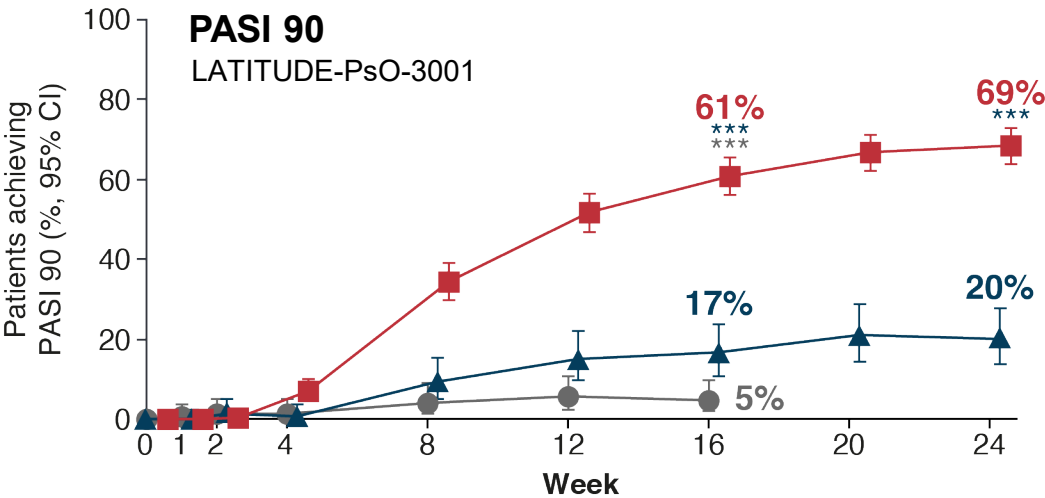
Zasocitinib met the co-primary endpoints in both studies (sPGA 0/1 and PASI 75 versus placebo at Week 16)



■ Zasocitinib ▲ Apremilast ● Placebo

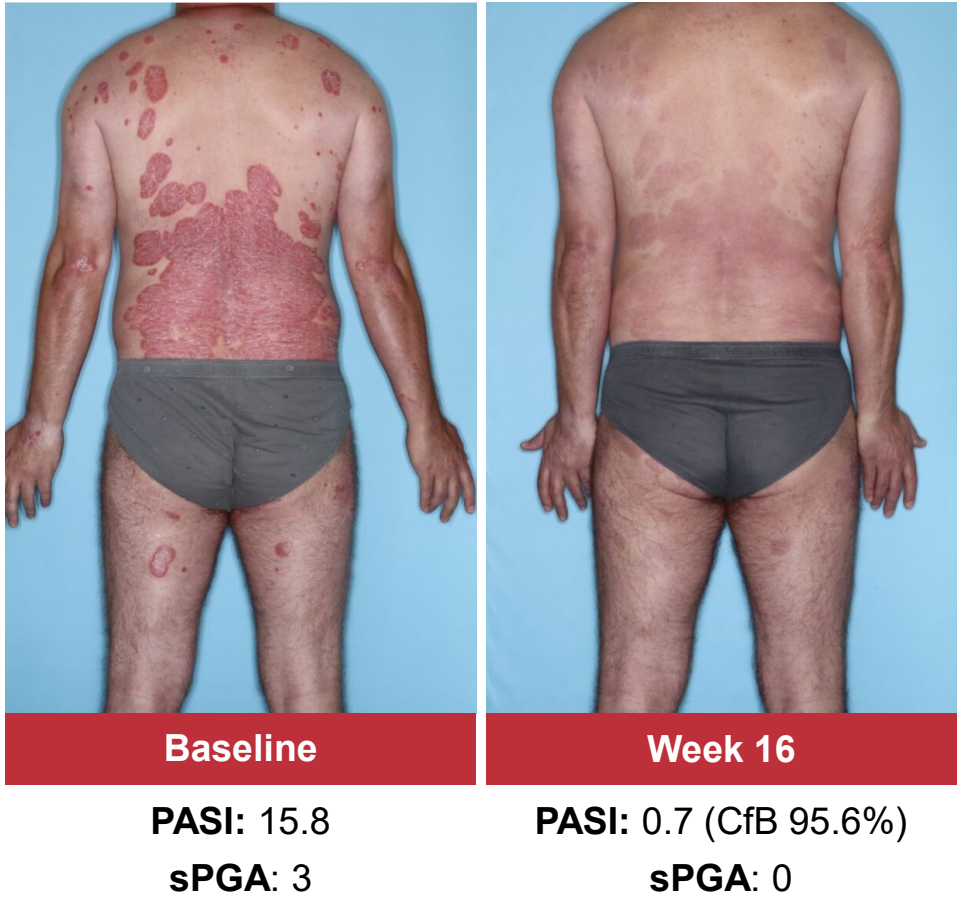
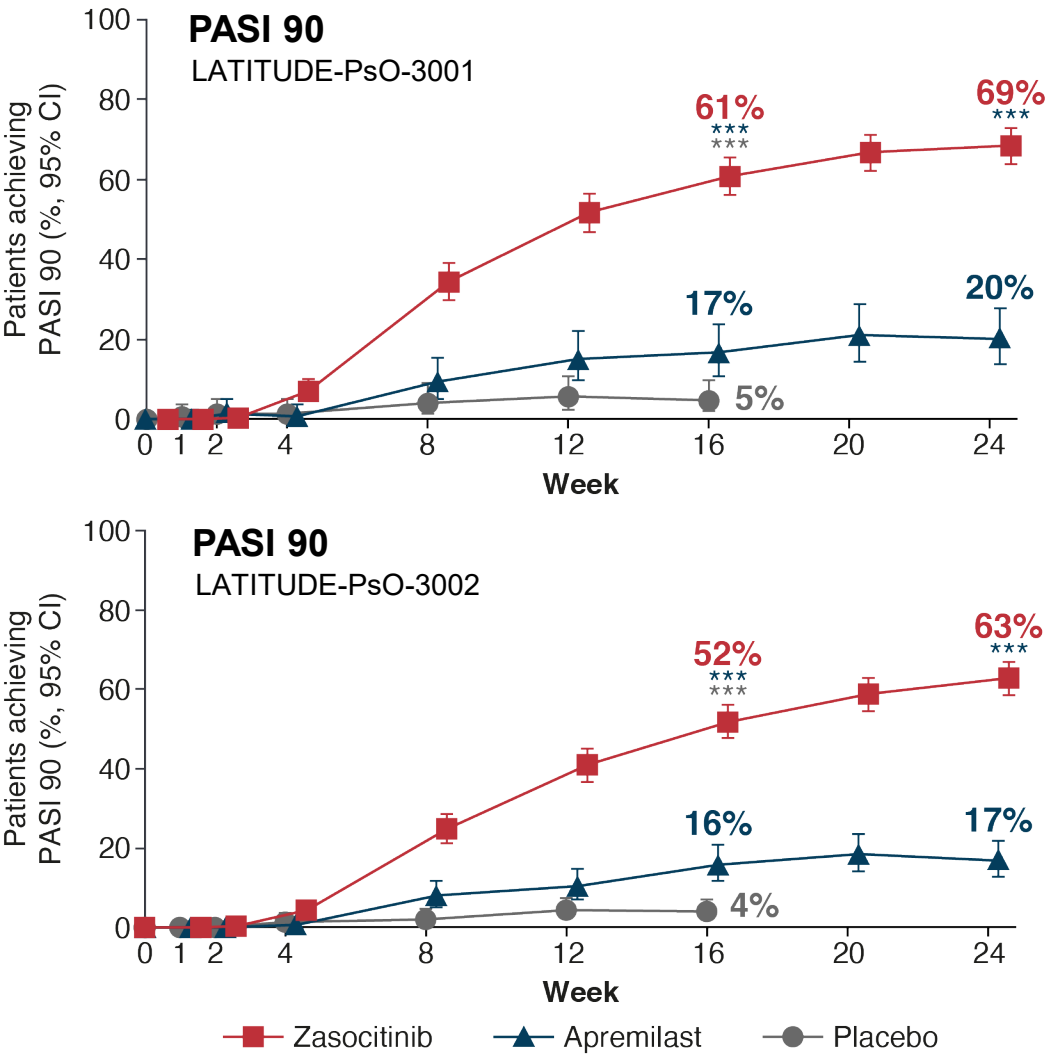
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Zasocitinib led to greater proportions of patients achieving PASI 90 than apremilast or placebo as early as Week 4



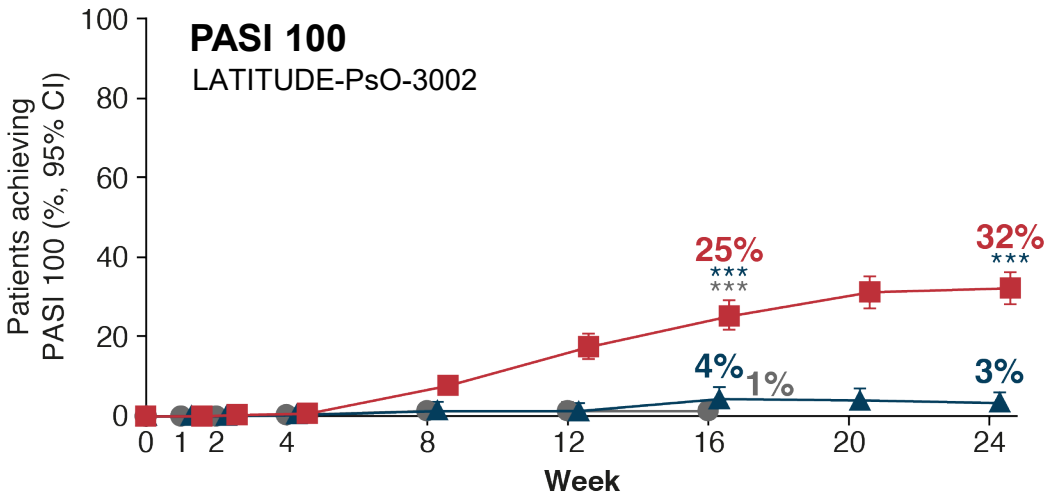
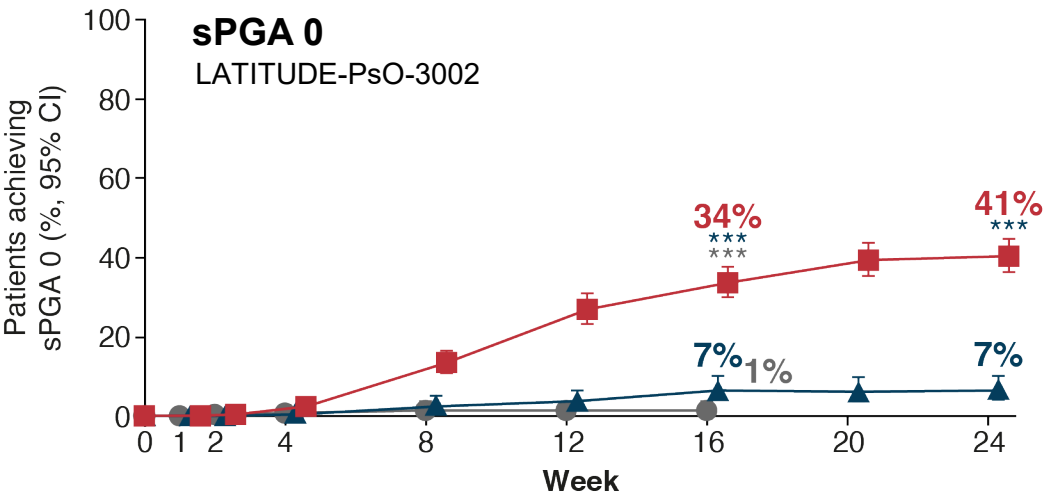
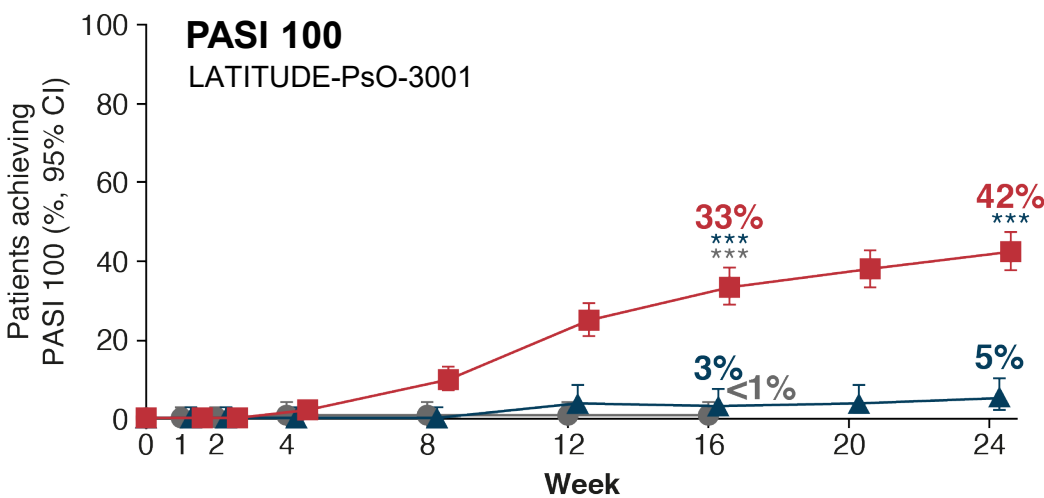
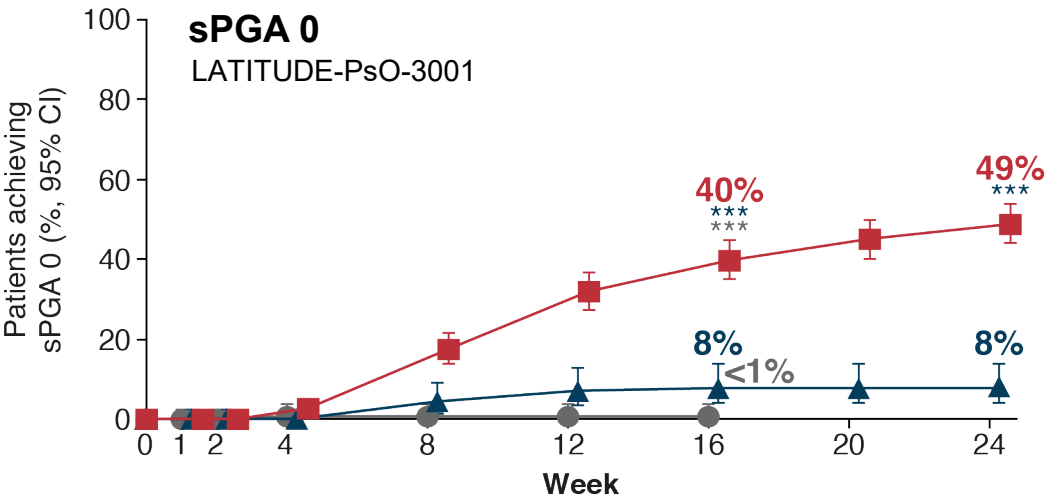
Number of patients based on the full analysis set with non-responder imputation. LATITUDE-PsO-3001: zasocitinib (n = 416), apremilast (n = 137), placebo (n = 140). LATITUDE-PsO-3002: zasocitinib (n = 555), apremilast (n = 276), placebo (n = 277). *P* values for comparison versus apremilast (in blue) and versus placebo (in gray) based on a stratified Cochran–Mantel–Haenszel test: ****p* < 0.001. Cfb, change from baseline; CI, confidence interval; PASI, Psoriasis Area and Severity Index; sPGA, static Physician's Global Assessment.

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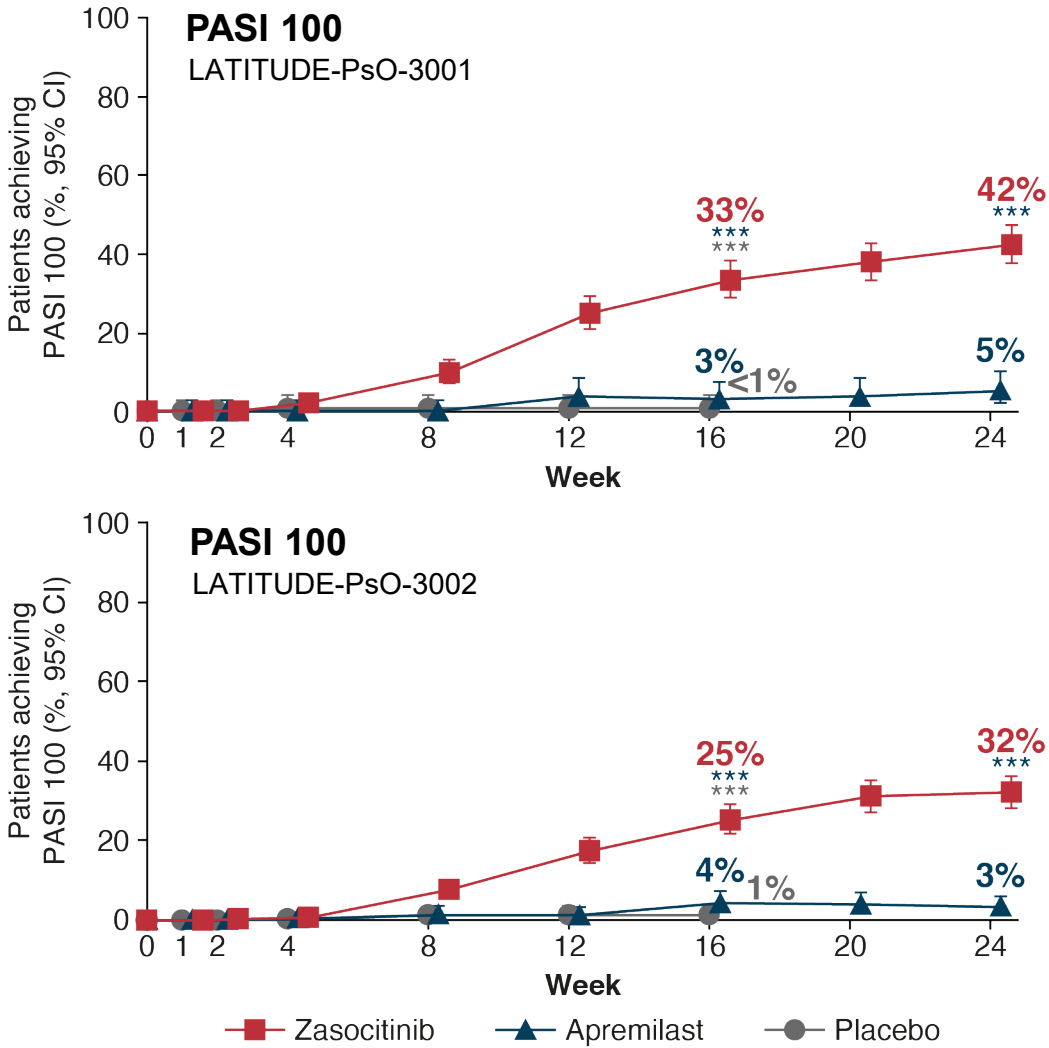
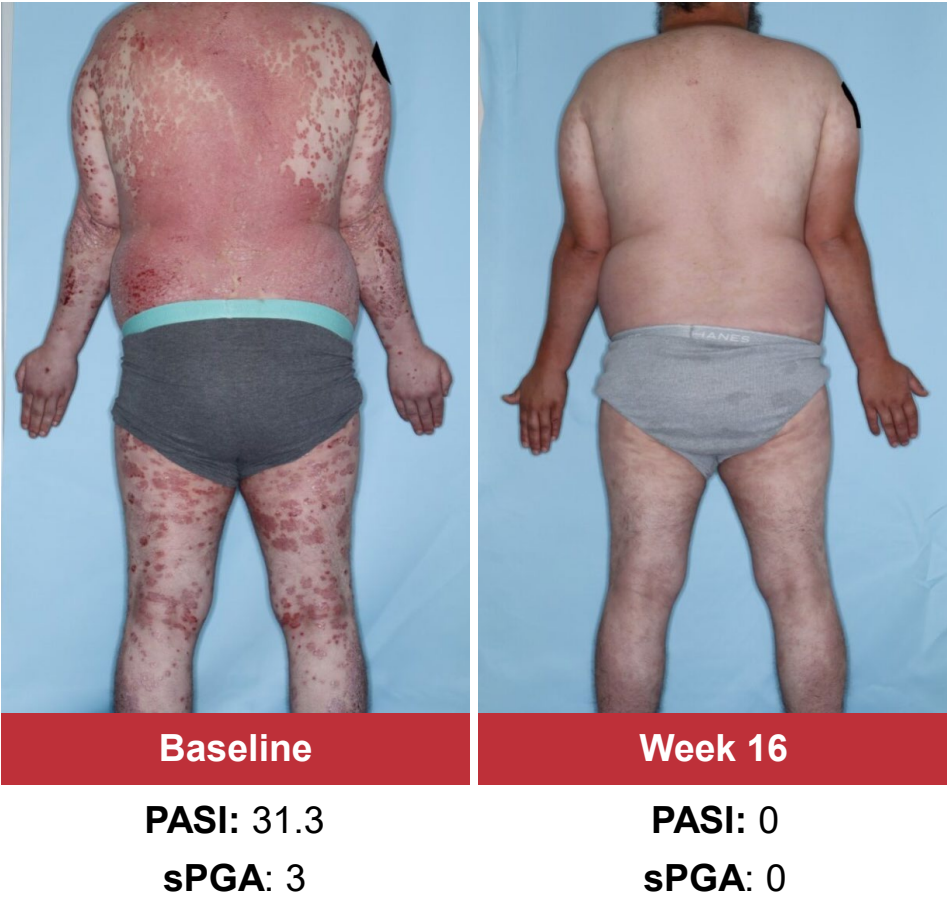
Zasocitinib led to greater proportions of patients achieving clear skin versus apremilast or placebo as early as Week 8



■ Zasocitinib ▲ Apremilast ● Placebo

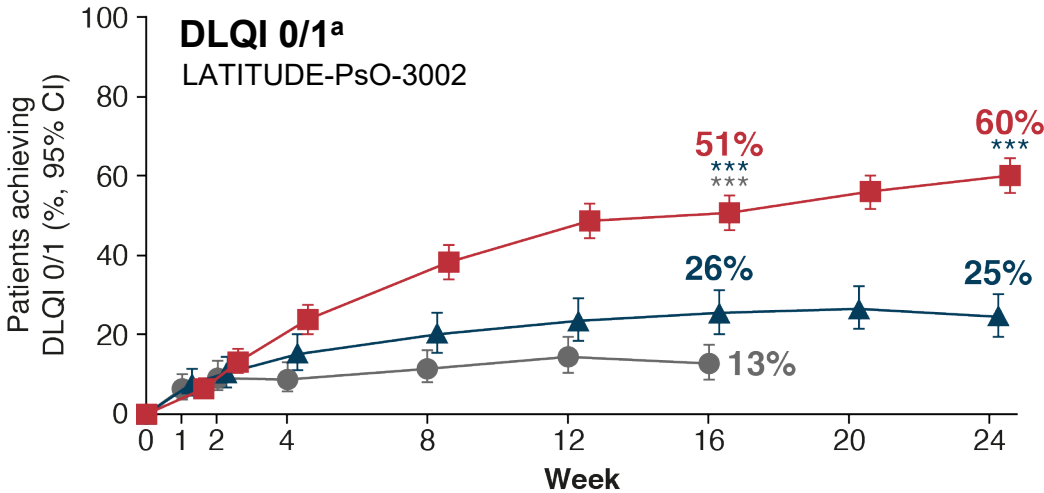
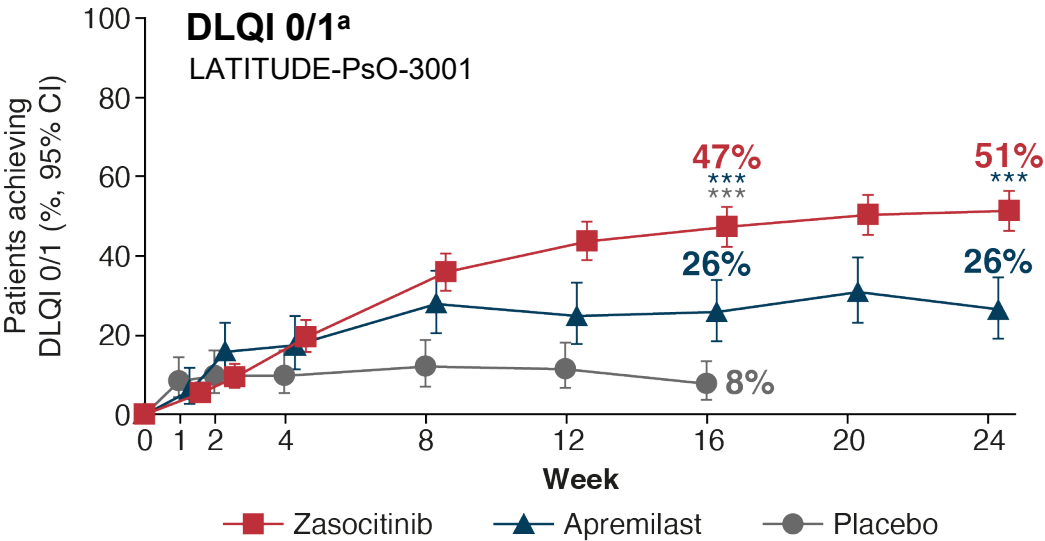
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Zasocitinib demonstrated superior improvement in DLQI versus apremilast or placebo as early as Week 4



^aBased on evaluable patients defined as a subset of full analysis set with a baseline DLQI score ≥ 2 (with nonresponder imputation). Number of evaluable patients for LATITUDE-PsO-3001: zasocitinib (n = 406), apremilast (n = 133), placebo (n = 133). Number of evaluable patients for LATITUDE-PsO-3002: zasocitinib (n = 525), apremilast (n = 263), placebo (n = 260). *P* values for comparison versus apremilast (in blue) and versus placebo (in gray) based on a stratified Cochran–Mantel–Haenszel test: ****p* < 0.001. CI, confidence interval; DLQI, Dermatology Life Quality Index.

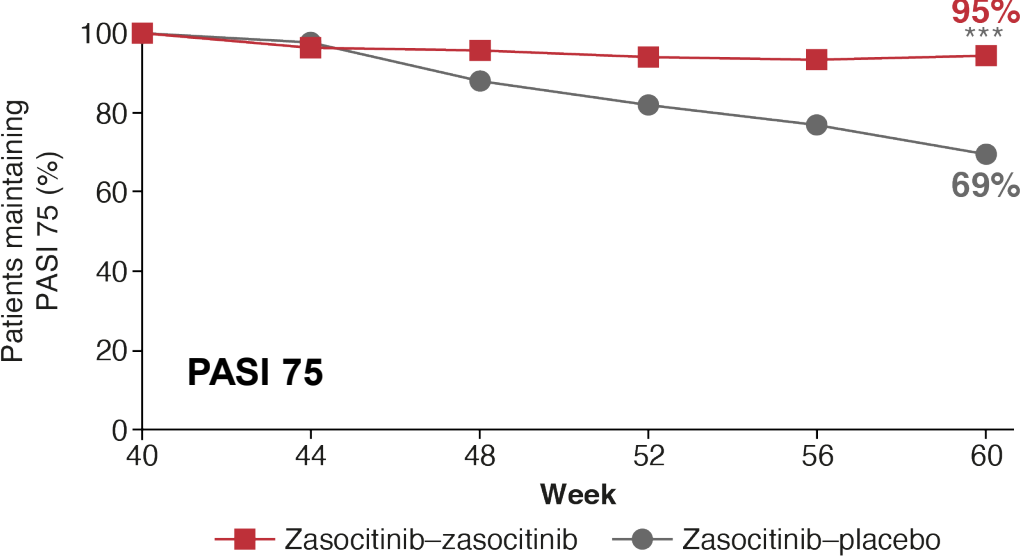
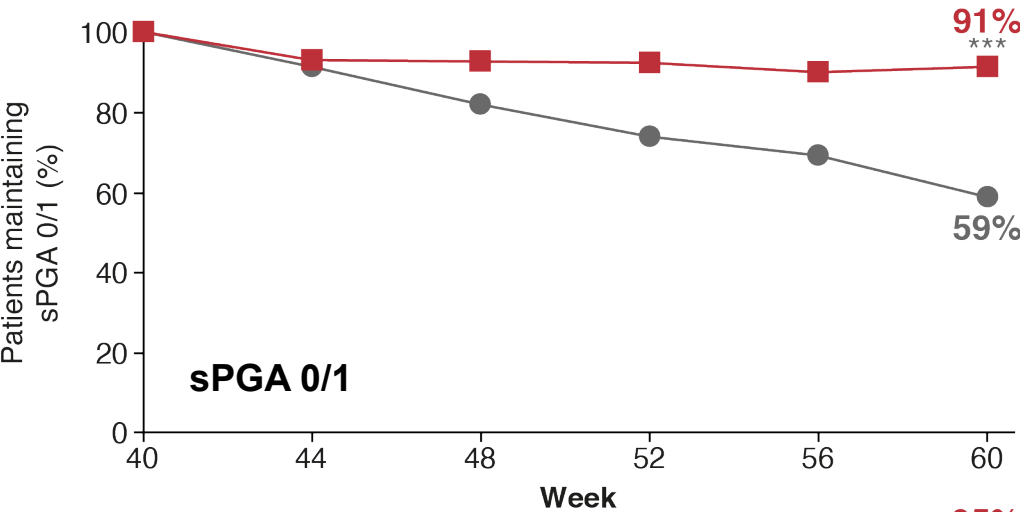
Zasocitinib was well tolerated with no new safety signals identified through Week 24

TEAEs from LATITUDE-PsO-3001 and 3002 ^a	Day 0 to Week 16						Day 0 to Week 24			
	Zasocitinib (n = 970)		Apremilast (n = 412)		Placebo (n = 417)		Zasocitinib (n = 970)		Apremilast (n = 412)	
	n	% ^b (95% CI) ^c	n	% ^b (95% CI) ^c	n	% ^b (95% CI) ^c	n	% ^b (95% CI) ^c	n	% ^b (95% CI) ^c
Any TEAE	605	62.1 (59.0–65.1)	207	50.5 (45.7–55.4)	196	46.9 (42.0–51.7)	674	69.3 (66.4–72.2)	232	56.5 (51.7–61.4)
Leading to discontinuation	31	3.2 (2.1–4.3)	11	2.6 (1.1–4.2)	3	< 1 (0.0–1.6)	36	3.7 (2.5–4.9)	13	1.3 (1.4–4.7)
SAE	29	3.0 (1.9–4.1)	6	1.5 (0.3–2.7)	2	< 1 (0.1–1.7)	35	3.6 (2.4–4.8)	7	1.7 (0.5–3.0)
Death	1 ^d	< 1 (0.0–0.6) ^d	0	0 (0.0–0.9)	0	0 (0.0–0.9)	1 ^d	< 1 (0.0–0.6) ^d	0	0 (0.0–0.9)
Most frequent TEAE (≥ 5%) ^e										
URTI	100	10.1 (8.2–12.0)	24	6.0 (3.7–8.3)	13	3.2 (1.5–4.8)	123	12.5 (10.4–14.6)	29	7.4 (4.8–10.0)
Acne	62	6.5 (5.0–8.1)	3	< 1 (0.0–1.7)	1	< 1 (0.0–1.3)	70	7.3 (5.6–8.9)	3	< 1 (0.0–1.7)
Nasopharyngitis	60	6.2 (4.7–7.7)	23	5.4 (3.2–7.5)	20	4.7 (2.7–6.6)	80	8.3 (6.5–10.0)	34	7.9 (5.4–10.5)
Diarrhea	30	3.1 (2.0–4.2)	33	8.2 (5.5–10.9)	8	1.8 (0.6–3.1)	36	3.7 (2.5–4.9)	33	8.2 (5.5–10.9)
Headache	27	2.8 (1.8–3.9)	26	6.3 (4.0–8.7)	8	1.9 (0.6–3.2)	32	3.3 (2.2–4.5)	28	6.8 (4.4–9.3)
Nausea	20	2.1 (1.2–3.0)	23	5.5 (3.3–7.8)	5	1.2 (0.1–2.2)	23	2.4 (1.4–3.4)	24	5.8 (3.5–8.1)

- Most TEAEs were **mild** or **moderate**
- **Laboratory parameters (e.g. lymphocytes, liver enzymes, lipids) demonstrated no clinically meaningful trends over time in both studies**

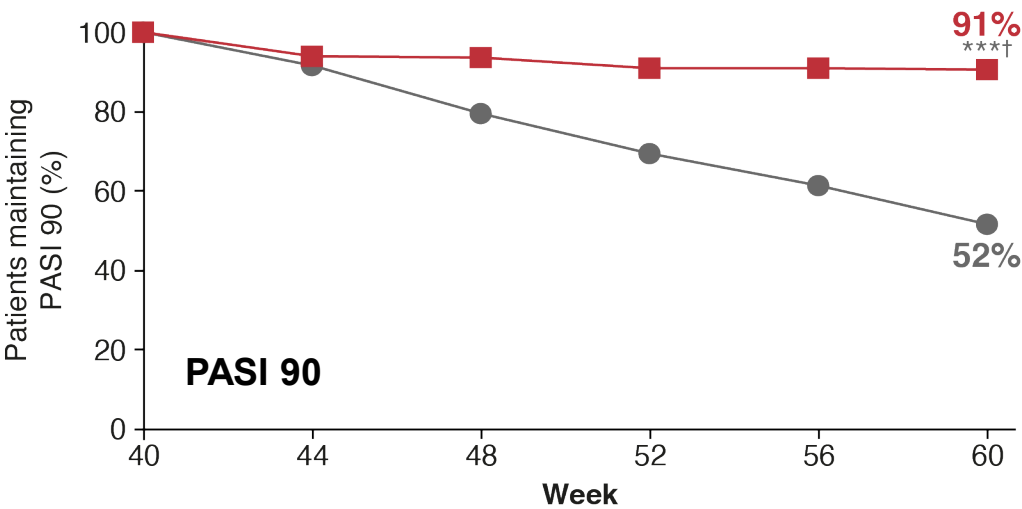
TEAEs were coded using MedDRA v28.1.
^aEvents starting while on initial treatment are included. ^bSample size adjusted incidence proportion x 100. ^c95% Wald CI unless 0 events occur in either trial, in which case a 95% exact binomial CI is used. ^dDeath occurred 1 day after first dose date (unrelated to treatment). ^eMost frequently reported adverse events occurring in ≥ 5% of patients in any treatment group, based on individual preferred term. CI, confidence interval; MedDRA, Medical Dictionary for Regulatory Activities; SAE, serious adverse event; TEAE, treatment-emergent adverse event; URTI, upper respiratory tract infection.

More than 90% of patients continuing zasocitinib at Week 40 maintained sPGA 0/1, PASI 75 and PASI 90 through Week 60



LATITUDE-PsO-3002 randomized withdrawal

Most (59%, 69% and 52%) patients re-randomized from zasocitinib to placebo at Week 40 maintained sPGA 0/1, PASI 75, and PASI 90, respectively, for an additional ~5 months



Evaluable patients based on the full analysis set for randomized withdrawal with nonresponder imputation. Number of patients for sPGA 0/1: zasocitinib-zasocitinib (n = 255), zasocitinib-placebo (n = 126). Number of patients for PASI 75: zasocitinib-zasocitinib (n = 273), zasocitinib-placebo (n = 134). Number of patients for PASI 90: zasocitinib-zasocitinib (n = 238), zasocitinib-placebo (n = 122). P values for comparison versus zasocitinib-placebo (in gray) based on a stratified Cochran-Mantel-Haenszel test: ***p < 0.001; ***† nominal p < 0.001. PASI, Psoriasis Area and Severity Index; sPGA, static Physician's Global Assessment.

Conclusions



Once-daily oral zasocitinib demonstrated rapid reproducible and durable skin clearance, and a consistent safety and laboratory parameter profile, across two pivotal phase 3 trials

- Clear skin was achieved by ~one-third of zasocitinib-treated patients by Week 16
- More than 90% of patients continuing zasocitinib at Week 40 maintained sPGA 0/1, PASI 75 and PASI 90 through Week 60
- Zasocitinib demonstrated superior improvement in QoL versus placebo or apremilast
- Zasocitinib was generally well tolerated with no new safety signals identified



Zasocitinib efficacy and safety will be further evaluated in patients with moderate-to-severe plaque psoriasis in a 3-year long-term extension study (NCT06550076) and an ongoing head-to-head trial versus deucravacitinib (NCT06973291)

Acknowledgments

We would like to acknowledge and thank all study sites and patients for their participation in this study

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